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PATIENT REPORT

Patient Age/Sex: **Female**

Specimen Collected: Not provided

DNA Extract and Hold	**|Received: 17-Jul-19 14:19**		**Report/Verified: 16-Sep-19 15:26**
Procedure	**Result**	**Units**	**Reference Interval**
DNA Extract and Hold	Complete [i1]		

TECTA N1474fs Variant Analysis	**|Received: 17-Jul-19 14:19**		**Report/Verified: 16-Sep-19 15:26**
Procedure	**Result**	**Units**	**Reference Interval**
TECTA N1474fs Variant Analysis	See Note [i2]		
Specimen	Whole blood (EDTA)		

Test Information

i1: DNA Extract and Hold
INTERPRETIVE INFORMATION: DNA Extract and Hold

This test was developed and its performance characteristics determined by Apricot Reference Laboratories. It has not been cleared or approved by the U.S. Food and Drug Administration.

i2: TECTA N1474fs Variant Analysis Int

INDICATION FOR TESTING: Congenital hearing loss. CLINICAL HISTORY: Deafness; Dizzy spell; Ringing in ears.

RESULT: NEGATIVE. The *TECTA* c.4422del variant was not detected.

INTERPRETATION: No reportable variant was found. The test looked for one known change in the TECTA gene. No reportable variant was found, so the change the test was designed to detect is not present. This result does not rule out hereditary hearing loss, because the test checks only one change. Other genes and other changes can cause hearing loss, so a broader test may be needed for a reason for testing of congenital hearing loss.

BACKGROUND INFORMATION: TECTA N1474fs Variant Analysis

CHARACTERISTICS: Meniere disease is an inner ear disorder. It causes episodes of spinning vertigo, fluctuating hearing loss, ringing in the ear and a feeling of fullness in the ear. The episodes can last from minutes to hours.

INHERITANCE: Most cases are not clearly inherited. A few families have several affected members, which suggests a genetic contribution.

CLINICAL SENSITIVITY: Varies with the condition and the population tested.

METHODOLOGY: Real-time PCR genotyping of the targeted variant with fluorescent probes.

LIMITATIONS OF ANALYSIS: Some pathogenic variants lie outside the coding regions of genes. The test may miss mosaic variants and variants in regulatory, deep intronic or highly homologous regions.

LIMITATIONS OF REPORTING: The report omits benign and likely benign variants.

Ask the care team to review the hearing history with a clinician who manages hereditary hearing loss, because a single-change test does not rule out other causes.

Ask whether a multigene panel or exome sequencing is appropriate if hereditary hearing loss remains a concern.

*=Abnormal, #=Corrected, C=Critical, f=Result Footnote, H-High, i-Test Information, L-Low, t-Interpretive Text, @=Performing lab